

LEGISLATURE OF THE STATE OF IDAHO
Sixty-eighth Legislature Second Regular Session - 2026

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 788

BY HEALTH AND WELFARE COMMITTEE

AN ACT

RELATING TO PRECEPTOR PRIOR AUTHORIZATION EXEMPTION; AMENDING CHAPTER 2, TITLE 56, IDAHO CODE, BY THE ADDITION OF A NEW SECTION 56-276, IDAHO CODE, TO ESTABLISH PROVISIONS REGARDING PRECEPTORS AND PRIOR AUTHORIZATION EXEMPTIONS; AND DECLARING AN EMERGENCY AND PROVIDING AN EFFECTIVE DATE.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. That Chapter 2, Title 56, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW SECTION, to be known and designated as Section 56-276, Idaho Code, and to read as follows:

56-276. PRECEPTORS -- PRIOR AUTHORIZATION EXEMPTION. (1) An eligible practitioner who provides three hundred sixty (360) hours or more of preceptorship in a calendar year, with at least sixty percent (60%) of those hours occurring in a qualified rural or underserved area, shall qualify for a twelve (12) month exemption from prior authorization requirements for medically necessary services billed to Idaho medicaid plans.

(2) (a) An eligible practitioner shall be responsible for tracking the eligible practitioner's preceptorship hours and may report such hours to the department of health and welfare upon reaching the minimum amount of hours needed to qualify for the exemption described in subsection (1) of this section.

(b) The department of health and welfare shall verify the hours submitted by an eligible practitioner, and upon verification shall exempt the eligible practitioner from prior authorization requirements as described in subsection (1) of this section at the start of the next calendar year.

(3) The department of health and welfare may rescind an exemption from prior authorization requirements if:

(a) On the basis of a review of a random sample of a statistically significant number of claims submitted by an eligible practitioner during an exempted period, the department determines that the eligible practitioner has fallen below the acceptable risk-weighted claim sampling score. Such a review may not be conducted more than once per year; and

(b) The department notifies the eligible practitioner at least twenty-five (25) days before the proposed rescinding from exemption is to take effect.

(4) No more than one hundred (100) eligible practitioners from each eligible specialty shall receive an exemption from prior authorization requirements pursuant to this section per year.

(5) The department shall promulgate rules, subject to legislative approval, to carry out the provisions of this section, including the develop-

1 ment of a risk-weighted sampling methodology and scoring system that incor-
2 porates both the number and dollar amount of claims.

3 (6) For the purpose of this section, the following definitions apply:

4 (a) "Eligible practitioner" means a physician, physician's assis-
5 tant, or advanced practice registered nurse practicing an eligible
6 specialty.

7 (b) "Eligible specialty" means family medicine, psychiatry, or obstet-
8 rics and gynecology.

9 (c) "Graduate student" means a medical, physician assistant, or ad-
10 vanced practice registered nurse student, resident, or fellow seeking
11 a degree at the graduate level at any accredited Idaho institution of
12 higher education, including Idaho-funded medical education programs,
13 or completing training in an Idaho-based residency or fellowship pro-
14 gram.

15 (d) "Preceptorship" means a mentoring experience in which a preceptor
16 provides a program of personalized instruction, training, and supervi-
17 sion that is offered to eligible graduate students to enable the stu-
18 dents to obtain professional degrees.

19 (e) "Qualified rural or underserved area" means a primary care health
20 professional shortage area or a medically underserved area, as those
21 terms are defined in section 39-5903, Idaho Code.

22 SECTION 2. An emergency existing therefor, which emergency is hereby
23 declared to exist, this act shall be in full force and effect on and after
24 July 1, 2026.